

ORIGINAL RESEARCH

HEART FAILURE

Standardized Reporting in Heart Failure Noninvasive Remote Monitoring Trials

Interventions to Catalyze Data Into Action



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ABSTRACT

BACKGROUND Noninvasive remote patient monitoring (RPM) captures and transmits physiological (eg, weight) and symptom data to the patient's care team for monitoring outside of the clinic, creating opportunities for early intervention (eg, medical therapy optimization) to avoid decompensation events. Mixed results in studies of noninvasive RPM may stem from the complex design of heart failure programs. In current RPM study reports, many workflow components including data review and clinical responses are reported ambiguously or not at all. Opaque workflows prevent robust evaluation, replication in other studies, and community implementation of heart failure RPM programs.

OBJECTIVES The purpose of this study was to create a set of recommendations and a template for reporting workflow design in the publication of noninvasive RPM studies.

METHODS The Heart Failure Collaboratory, Canadian Heart Failure Society, and members of the European Society of Cardiology formed a multidisciplinary working group. Through an extensive consensus-building process, the authors reviewed existing workflows and data reporting practices and developed "the 6 Rs" of RPM Reporting.

RESULTS The authors created a set of recommendations and a template for reporting workflows that is organized around 6 Rs: patient Representativeness, ambulatory data Recording, data Relay to clinicians, clinician data Review, Response, and Recommendations from patients and providers.

CONCLUSIONS Systematic use of the 6 Rs of RPM Reporting template will improve the completeness and transparency of workflow reporting of RPM studies. The authors encourage investigators to use this framework and accompanying template during noninvasive RPM trial planning and include completed templates in study publications or as supplemental materials. (JACC Heart Fail. 2026;14:102849) © 2026 The Authors. Published by Elsevier on behalf of the American College of Cardiology Foundation. This is an open access article under the CC BY-NC-ND license (<http://creativecommons.org/licenses/by-nc-nd/4.0/>).

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**ABBREVIATIONS
AND ACRONYMS****AI** = artificial intelligence**BP** = blood pressure**EHR** = electronic health record**GDMT** = guideline-directed
medical therapy**HF** = heart failure**PAD** = pulmonary artery
diastolic**RPM** = remote patient
monitoring

Despite improvement with current guideline-directed medical therapy (GDMT), rates of heart failure (HF) hospitalization and mortality remain high.¹ Although acute decompensated HF can occur abruptly, deterioration often occurs gradually.² Maintaining intracardiac filling pressures below the congestion threshold along with education, lifestyle modifications, diuretics, and vasoactive drugs may prevent clinical deterioration in HF patients.³ Remote patient monitoring (RPM) captures and transmits disease-

relevant patient data to care teams without face-to-face encounters. With frequent data sharing, RPM may enable opportunities for increased monitoring and early intervention, and lead to patient empowerment, improved quality of life, and personalized care.⁴⁻⁸

Several factors including the COVID-19 pandemic, emerging digital health care platforms, patient preferences, and pressure to improve cost efficiency have accelerated the transition away from in-person care and toward RPM and telehealth.^{8,9} Although digital health interventions have the potential to improve access to care, they risk exacerbating disparities if implementation is inconsistent or inequitable. A systematic framework for reporting RPM studies is critical for evidence-based development and deployment of technologies and to promote transparency, enhance replicability, and guide the equitable adoption of RPM across diverse, underserved, and resource-limited settings. Invasive pressure monitoring systems¹⁰⁻¹² appear effective in optimizing intracardiac filling pressures and improving clinical endpoints but are not widely used due to cost, access, and infrastructure requirements.^{12,13} Noninvasive devices (eg, scale, wearables) are accessible to broader populations without indwelling hardware or significant technical requirements. Interventions may target clinician review and response or patient self-response to RPM data. Patient self-response introduces significant complexities and challenges. Given these considerations, we focus herein on noninvasive RPM interventions that target clinician review and response, although studies may also incorporate elements of patient self-correction.

A recent meta-analysis suggested that noninvasive RPM reduces all-cause mortality and HF hospitalizations,¹⁴ but there remains considerable variability in study outcomes. Estimates of cost effectiveness of HF RPM are varied due to many factors including national infrastructure, payor models, and local

clinical and patient features.¹⁵⁻¹⁷ RPM workflows are sophisticated (**Figure 1**), raising the importance of comparing components of RPM workflows to guide refinement, iteration, and development of sustainable interventions that can be translated into community practice and optimize available resources (**Figure 2**). Nevertheless, a survey of 63 noninvasive HF RPM studies found that many aspects of workflows are reported ambiguously or not at all, preventing robust evaluation, replication, and community implementation. Key examples include clinician availability for data review, communication format during clinical escalation, and response time to alerts (inadequately reported in 67%, 75%, and 89% of studies, respectively).¹⁸

The HFC (Heart Failure Collaboratory) is a public-private partnership with the United States Food and Drug Administration aimed to improve the ecosystem of HF clinical trials, including the use of digital tools to enhance trial efficiency. The HFC partnered with the Canadian Heart Failure Society and members of the European Society of Cardiology to develop a framework for reports of noninvasive HF RPM studies to streamline reporting, evaluation, and comparison of strategies. This framework is intended to standardize reporting of trials, not suggest requirements for study design.

Our framework (**Central Illustration, Figure 3**) for the comprehensive reporting of workflows and trial design is summarized through “the 6 Rs” of RPM reporting: patient Representativeness; ambulatory Recording; Relaying data to clinicians; Review of data; clinical Response to actionable data; and Recommendations from patients and providers. We encourage investigators to use this framework and accompanying template (**Supplemental Table 1**)¹⁹ during noninvasive RPM trial planning and include completed templates in study publications or as supplemental materials. To illustrate implementation of the reporting template, we provide completed templates for 2 trials with mixed results (**Supplemental Tables 2 and 3**).^{20,21}

FRAMEWORK DEVELOPMENT

We reviewed HF RPM studies to analyze existing workflows and data reporting practices.¹⁸ From this review, we identified key aspects of RPM implementation, synthesized them into a preliminary checklist, and presented it to the multidisciplinary working group, which included academia, industry, regulatory experts, and patient partners with lived experience. We did not use a structured consensus method but instead engaged through a series of

FIGURE 1 RPM Programs are Highly Complex From Enrollment to Study Closure



Each aspect of RPM programs contains multiple components that may introduce many decision points and potential for significant variation between studies of similar aim. RPM = remote patient monitoring.

workshop and discussion sessions to systematically evaluate each checklist item to identify gaps, clarify definitions, and categorize components into overarching domains. This extensive consensus-building process led to the development of the 6 Rs of RPM Reporting.

THE 6 Rs OF RPM REPORTING

REPRESENTATIVENESS. Generalizability is largely determined by population representativeness and extrinsic factors such as implementation pathways. Representativeness in RPM trials likely impacts effectiveness and equity more than for other HF interventions. The importance of representativeness applies to patients and participating providers because each role individually and the interaction between them can impact the RPM workflow. Sub-group analyses of clinical trials for pharmacologic and device therapies often demonstrate similar relative risk reduction (and sometimes greater absolute risk reduction) in patients with comorbidities. This may reflect a direct physiological effect on the failing heart. By contrast, noninvasive RPM primarily supports behavioral interventions by coordinating multifaceted aspects of clinical care such as timely evaluation and intervention, care access and coordination, and GDMT optimization. The foundation of RPM is the repeated evaluation of signals whose accuracy changes and generalizability are compromised if the spectrum of trial patients is dissimilar to those monitored in clinical practice (spectrum bias).²² Once deployed, the level of efficacy is challenging to quantify and may be different than the primary study. Monitoring performance will also depend on

population characteristics, such as education, socioeconomic factors, demographics, comorbidities, and patient preferences.

RPM trial inclusion criteria should be broad with limited exclusion criteria. Addressing challenges associated with recruitment for complex RPM trials remains out of scope for this statement, but improved reporting of eligibility criteria, outreach methods, and participant representativeness will deepen understanding of barriers to participation and inform future enrollment strategies. Trials should report methods used to identify the target population, screening numbers, proportion of subjects meeting inclusion and exclusion criteria, reasons for exclusion, proportion and reasons for declining participation, characteristics of those who declined participation, full withdrawal, loss to follow-up overall and for specific endpoints, intervention discontinuation, and standardized reasons for discontinuation.²³ The contact and consent process for RPM trials should aim to minimize selection bias and include marginalized communities. For low-risk interventions, an opt-out consent approach increases overall uptake and inclusion of female and non-White participants.²⁴ The method of consent (eg, mail, phone), levels (eg, tiered methods), message content, timing, and rates of opt-outs should be reported. Ideally age, sex, race, ethnicity, education, technological literacy, insurance, income, socio-demographics, comorbidities, and medical therapy for enrolled participants, participants who drop out, and those who declined to participate should be reported. Health service planners and providers can use this information to estimate potential uptake and reach.

FIGURE 2 Potential Impact of Study Design Variations on Outcomes

(A) The Chaudhry et al³⁸ study reviewed RPM patient data daily during business hours and observed no change in all-cause mortality. (B) The Koehler et al²¹ study had data reviewers available 24/7 and observed decreased all-cause mortality. Improved study reporting will allow more systematic analyses to determine which aspects of workflow underly differences in outcomes. Abbreviation as in [Figure 1](#).

RECORD. Evaluation of RPM interventions is contingent on consistent reporting of elements describing the remote collection of patient-recorded data. Key elements may vary by study and depend on local clinical conditions. Recently discharged patients after an acute HF hospitalization or those who experience changes in symptoms may require more frequent collection of data, whereas stable patients undergoing GDMT optimization may require less frequent monitoring.²⁵ The cadence of data capture, changes during the study, and basis for these decisions should be reported. Symptoms may be captured as subjective indicators of a patient's health status. Although we suggest time-stamped entries should be characterized with respect to severity, duration, and any precipitating or alleviating factors, in 24% of studies the method of symptom collection was unclear and 8% did not clearly describe the timing of collection.¹⁸ Physiological data measurements, modality used for measurements, and device validation and calibration should be specified.

There is high concordance in devices used by RPM studies; 95% of studies provided scales and 76% provided blood pressure (BP) cuffs to patients.¹⁸ To our knowledge, few studies, if any, report the calibration procedures used for such devices.¹⁸ Given variability in the precision of different devices (eg, brachial vs radial artery BP cuff), make and model of the device, patient position during measurement, and protocol recommended to the patient to improve accuracy should be reported (eg, number of readings taken, time interval between them). The use of wearable devices, whether clinical or commercial, in RPM studies has not yet been defined,²⁶ although it is likely the principles outlined earlier will remain applicable.

Some data suggest that RPM technologies are accepted by patients with HF,²⁷⁻²⁹ but patients with HF are typically older and the home setup requirements may represent a barrier to uptake, reduce compliance, and decrease accessibility. Requirements for support from family, medical

FIGURE 3 Highlights

1. Noninvasive RPM has the potential to improve HF outcomes, however, published studies often lack sufficient details of workflow design.
2. The HFC, CHFS, members of the ESC formed a multidisciplinary working group and developed the 6 Rs of RPM Reporting: a standardized structure for reporting workflows in HF RPM trials.
3. The 6 Rs of RPM reporting are patient Representativeness, ambulatory Recording, Relaying data to clinicians, Review of data, Response to actionable data, and Receiving recommendations from patients and providers.
4. Representativeness describes the study population and impacts generalizability to ensure findings apply to real-world settings.
5. Recording refers to remote collection of patient data, including physiological measurements and symptoms.
6. Relaying involves transmitting patient RPM data to clinicians and outlines patient and technical resources required for success.
7. Review describes how RPM data is aggregated, presented, and communicated to care teams.
8. Response captures clinical actions taken based on RPM data to demonstrate how RPM enables intervention and prevents deterioration.
9. Receiving recommendations reflects end-user engagement, including patients, clinicians, and support staff, and is key to RPM uptake and sustainability.
10. Using the 6 Rs framework enables comparison across RPM trials and supports real-world implementation.

Investigators are encouraged to use this framework and accompanying template during noninvasive RPM trial planning and include completed templates in study publications or as supplementary materials.

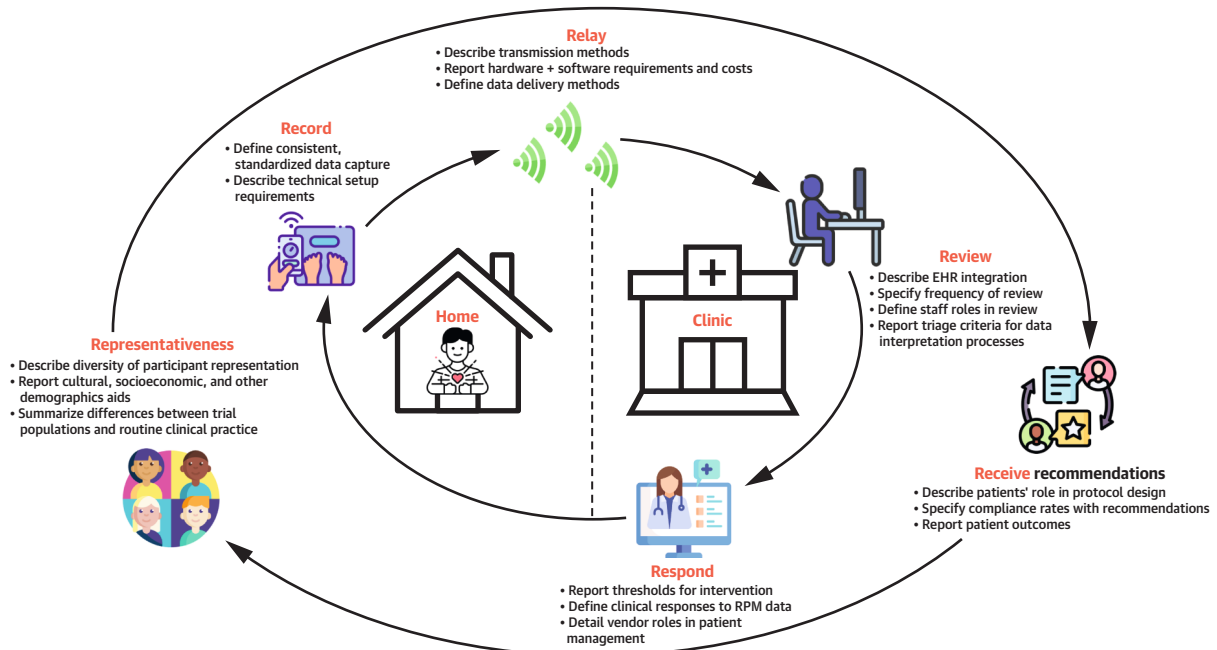
CHFS = Canadian Heart Failure Society; ESC = European Society of Cardiology; HF = heart failure; HFC = Heart Failure Collaboratory; other abbreviation as in [Figure 1](#).

professionals, technology specialists, or vendors, both at enrollment and longitudinally, should be reported to pre-empt these barriers.

RELAY. The modality used to transmit RPM data to the clinical team may be consequential for patient participation, engagement, compliance, and accuracy. There is variability in the methods used to transmit RPM data, with no more than 25% of published studies using the same method (Bluetooth).¹⁸ As technological innovation accelerates, heterogeneity in data transmission approaches is likely to persist, if not proliferate. Interconnected devices using technologies such as Wi-Fi and Bluetooth may facilitate more frequent measurements by reducing or eliminating patient effort required to transmit data. Dependence on increasingly complex digital technologies could limit access for patients with poor technical literacy, limited experience or access to the internet, or financial resources required for optimal device function. Technical requirements for patient participation must also be reported. Examples include landline vs smartphone, direct messaging (SMS [Short Message Service] vs MMS [Multimedia Message Service]), e-mail, home internet access, and bandwidth

(continuous/on-demand). Time requirements must also be articulated. Examples include patient-initiated contact (eg, scheduled phone calls or text messages to report data) or engagement with the monitoring device (eg, stepping on a Bluetooth-connected scale every morning). The total actual cost to the patient outside of the research study context should be summarized as a key indicator of access.

Research-driven interventions often leverage study-specific applications, enhanced monitoring protocols, or dedicated care teams that may differ from those available in routine clinical care or may not be scalable in standard practice. Given that there are currently no standards requiring minimum requirements for study applications, systematic description of study applications that are used will be important to develop such future standards and determine optimal methodologies. The mechanism of data delivery to the care team must also be stated. Examples include manual documentation of voice-voice phone calls, storage of free-text messages, data entry into a third-party platform, or interface with an electronic health record (EHR) to structured or unstructured data fields. Where applicable, it is important to describe software version,

CENTRAL ILLUSTRATION The 6 Rs of HF RPM Reporting

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Comprehensive and standardized reporting of workflows in HF RPM trials can be organized by the 6 Rs—patient Representativeness, ambulatory Recording, Relaying data to clinicians, Review of data, Response to actionable data, and Receiving recommendations from patients and providers. Each “R” is multifaceted, and trials should clearly report all aspects of design. EHR = electronic health record; HF = heart failure; RPM = remote patient monitoring.

interoperability capabilities, data encryption standards, care team technical requirements, and provider interface accessibility. These features have significant implications for program efficacy and implementation into clinical practice in terms of the resources necessary for replication.

REVIEW. Although there is select guidance available,³⁰ data review practices are variable with no clear consensus on how best to use resources for RPM interventions. The method for data aggregation and presentation to care team members to support real-time clinical decisions and long-term study outcomes must be described. RPM generates tremendous quantities of data that may overwhelm standard data review methodologies and augment the “productivity paradox” of health care digitization, wherein a greater volume of higher quality data paradoxically decreases provider efficiency. Efficient review strategies to identify clinically meaningful and actionable events among significant noise are therefore critical.³¹ A systematic review of RPM identified that factors such as rapid response times, high-risk patient targeting, accurate detection of

events, and provision of personalized and coordinated care improve effectiveness.³⁰

Although 51% of published studies used a registered nurse as the primary reviewer, the remaining were divided among 8 distinct mechanisms. Review frequency, specialty training of the primary reviewer (eg, HF-training), and review time were often unspecified (48%, 65%, and 67% of studies, respectively).¹⁸ The role, level of training, and scope of practice of each person that touches a patient’s data should be stated. The frequency of manual data interrogation or validation of automated data reviews, the interoperability of the data, and whether it is integrated into the EHR or is hosted by a third party (eg, vendor website) should be indicated. The complete workflow for event identification and alerting, including alert modalities (eg direct messaging vs EHR-based clinical decision support alerts), should be delineated. Data triage by automated systems can help reduce personnel workload by categorizing most data as nonactionable and highlighting trends or events that require personalized response or clinician input. If automated systems or clinical decision

support are used, the evidence base for trigger creation and cutoff formulation should be cited and reported. Finally, use of any monetary incentives for patients and staff and billing and payment workflow should be reported. Solutions chosen in each health care setting to optimize these factors will depend on the size of the health system, resources available to the team, the RPM technology being used, and the methods available for billing and reimbursement.³¹

RESPOND. Understanding of the “response element” in RPM studies constitutes a critical bridge between prediction of decompensation and successful prevention of deterioration. Poor alignment between the reimbursement of RPM and its potential value elevate the importance of robust evidence outlining the level of expertise and clinical response.³² Articulating response elements and associated outcomes may also help discern which aspects of patient RPM risk profiles are modifiable through different interventions, such as reassurance, medication titration, diuretic adjustments, and clinic visits, vs patients whose risk may not respond to HF-specific interventions (eg, sepsis, pain, chronic obstructive pulmonary disease). Several elements of the clinical response to RPM that merit reporting include: 1) threshold determination for what constitutes escalating clinical risk; 2) transparency in the process for conveying an escalation to clinical teams; 3) delineation of the clinician responsible for executing the clinical intervention (eg, study team, cardiologist); and 4) selection of clinical intervention (ie, algorithmic or left to the clinical judgement of the treatment team).

Alert thresholds may be dichotomous (ie, alert zone, nonalert zone) or continuous.^{33,34} Calibrating sensitivity and specificity of the system for worsening HF will distinguish RPM that provides a reliable signal of impending decompensation vs another technological tool with limited clinical uptake due to alert fatigue. High-volume data can enhance monitoring but risks clinician “alert fatigue,” potentially undermining provider well-being. Successful RPM will ultimately require novel technologies such as artificial intelligence (AI)-based RPM data monitoring and triage to ensure that the increased patient-staff ratios that may be possible with RPM are manageable and safe, but these technologies remain nascent. Strategies to mitigate provider fatigue and burn-out should be reported to help produce concrete evidence regarding implementation strategies that enhance care without overburdening clinicians.^{33,34} Response protocols should include mechanisms that allow opportunities for iterative review and

refinement of interventions incrementally as new signals emerge.

Expectations regarding response time by the initial data reviewer and performance as a continuous time measure should be reported including success of patient contact, number of attempts, modality of communication (eg, the use of scripts), opportunities for bidirectionality, and escalation instructions for staff. Staff roles in response to the alerts should be categorized, such as selection of ongoing monitoring, and setting time frames for specific follow-up, diagnostic testing, initiation and identity of therapies including patient self-management changes, or any decision-making that potentially identifies the alert as an error or false-positive. All contacts for any patients in a control group, if present, should be similarly catalogued as with the intervention group to enable adjusted comparisons, including allowing contact and intervention rate calculations by treatment arm. Although essential components, 11% of HF RPM studies did not report their escalation strategy. Among those that did, protocols varied significantly. Most did not report how reviewers contacted clinical personnel or response times to alerts (75% and 89%, respectively).¹⁸

Although the focus of this template is noninvasive devices, several important insights from invasive and wearable RPM device studies are applicable. A sub-analysis of CHAMPION (CardioMEMs Heart Sensor Allows Monitoring of Pressure to Improve Outcomes in Class III HF) showed that RPM with the CardioMEMs system led to intensification of GDMT in response to elevations in pulmonary artery diastolic (PAD) pressures.³³ Investigators were provided a “treatment algorithm” guiding response to low PAD (reduction in diuretics), normal PAD (no change), and elevated PAD (increase diuretics), but little guidance was provided regarding critical elements such as medication dose changes or how to use contemporary GDMT in concert with diuretic agents based on other physiological parameters (eg, BP, heart rate). Uncertainty regarding effective workflows and interventions has contributed to the underuse of PAD monitoring technology and suggests that detailed specification of algorithm, staffing requirements, and clinician response is important to report in noninvasive RPM. Personalized baselining and advanced analytics, as demonstrated in the LINK-HF (Multi-sensor Noninvasive Remote Monitoring for Prediction of HF Exacerbation) study³⁵ appears critical for risk prediction. As important as identifying impending risk may be the reliable prediction of patients with low HF risk to allow health systems to concentrate efforts on the subset of patients who remain at

risk. Individualized response plans to multimodal RPM may add a layer of complexity that must be reported to enable reproducibility and scalability.

Rigorous investigation is needed to understand how multiparameter technologies can be sustainably incorporated into clinical workflows. Several companies are pursuing the goal of aggregating, sorting, dashboarding, and modeling RPM data. Their broad use has been limited by lack of evidence, regulatory uncertainty, and challenges with integration into health care systems, especially the EHR. RPM programs must describe how devices and dedicated clinical support integrate with the local care team, particularly if external clinical support can contact patients or make treatment changes. Reporting of personnel, modality, timing, and degree of discretion involved in the intervention of RPM programs is paramount for health system leaders to understand the investment needed to support RPM and to project cost savings from improved quality metrics (eg, HF readmissions). Without data to support operational decisions, RPM may be relegated to the research realm without broad use in HF management.³⁵

RECEIVE RECOMMENDATIONS. Receptivity and engagement of end-users (primarily patients and health care providers) is paramount to the success and sustainability of RPM interventions. Trials must document the determinants and extent of end-user engagement, from the initial design to study analysis stages. Purposeful involvement of diverse end-user populations during the design phase is essential, ensuring the development of protocols that are realistic, implementable, and conducive to sustained engagement across a wide population with minimal disruption of established workflows. End-user involvement may foster a sense of ownership, empowerment, and partnership between patients and their care teams. Thorough documentation of end-users' involvement in decision-making processes, such as selecting relevant outcome measures, determining the frequency and feasibility of data transmissions, or cadence of data review, can enhance the relevance and acceptability of RPM interventions. Patient experience and response to recommendations from the care team must be articulated to determine whether observations from the RPM data led to actual changes in patient management. Use and compliance rates provide important metrics for evaluating the feasibility, effectiveness, and opportunities for improvement of RPM interventions. High adherence to monitoring protocols suggests acceptance of the RPM intervention and potential impact on end-user behavior and

outcomes. Conversely, low compliance rates may signal the need to modify the intervention design to enhance engagement and retention. Low compliance rates may be amplified when interventions are applied at scale or implemented in the community if not properly addressed and reported in clinical trial results.

Robust descriptions of advocacy and partnership initiatives are critical. Without early end-user involvement, an RPM program may be built only to find it is unusable for the target populations. Collaborations between health care organizations, patient advocacy groups, technology developers, and policymakers can help raise awareness about the benefits of RPM, address barriers to implementation, and advocate for policy changes to support widespread adoption. Researchers should describe how they leveraged collective expertise and resources from a variety of sectors to overcome challenges in implementation.

Formal implementation science frameworks, such as PRISM (Practical Robust Implementation and Sustainability Model),³⁶ offer valuable guidance for structured planning, executing, and evaluating RPM initiatives. These frameworks emphasize the importance of addressing contextual factors, stakeholder engagement, and intervention fidelity to achieve successful implementation and long-term sustainability. Comprehensive capture of the study characteristics described earlier will enable implementation scientists and health services researchers to apply these principles to enhance the effectiveness and scalability of RPM interventions.

OTHER CONSIDERATIONS

Broader contextual and operational factors beyond the scope of study design influence the implementation and outcomes of RPM initiatives. Examples include organizational readiness, resource availability, regulatory considerations, and cultural factors. RPM interventions will likely need to operate across diverse health care settings including community-based care, acute management, and multidisciplinary specialty clinics. Identification of such factors and their effects on execution of the RPM intervention may help overcome implementation barriers and maximize the impact of RPM interventions in community adoption. Many of the processes including those detailed within Record, Relay, and Review could occur synchronously—preferably through automation—and be iteratively refined based on clinical responses and outcomes, leveraging adaptive technologies such as AI where

appropriate. Integration of AI-based model predictions into RPM will become increasingly common as technological development advances RPM through novel wearables and implantable devices (eg, continuous glucose monitors).³⁷ Clear documentation of collaboration between vendors and research teams ensures transparent communication of technological aspects and visibility into potential conflicts of interest. Finally, certain parameters within RPM interventions will vary based on study endpoints and anticipated duration. Short-term studies may necessitate more frequent data transmission, whereas long-term assessments might adopt less intensive monitoring schedules given the longer timeline of demonstrable benefit. Understanding these dependencies is crucial for effective study design, interpretation, and generalization.

CONCLUSIONS

Through increased monitoring and early intervention, RPM holds great promise in transforming HF management by potentially improving patient outcomes and reducing health care costs. Comprehensive documentation of trial design is critical to realize this potential. We propose that systematic reporting of RPM studies using the 6 Rs is essential to evaluate and define optimal use of RPM: patient Representativeness; ambulatory Recording; Relaying data to clinicians; clinician Review of data; clinical Response to actionable data; and Recommendations from patients and providers (**Central Illustration, Supplemental Table 1**). Using this structured reporting model will support robust comparisons between different methodologies, including understanding of which components contribute to the success or failure of RPM programs and providing comprehensive evidence to translate clinical trial findings into real-world clinical implementation.

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PERSPECTIVES

COMPETENCY IN PATIENT CARE AND PROCEDURAL

SKILLS: The 6 Rs of RPM Reporting framework offers clinicians a practical tool to evaluate the design and applicability of noninvasive RPM interventions in HF. By standardizing reporting, it can help clinicians interpret evidence more effectively and identify strategies suited to their patient populations. This framework supports the integration of RPM into clinical workflows, with the potential to improve outcomes and reduce care delivery burden.

TRANSLATIONAL OUTLOOK: The 6 Rs of RPM Reporting work addresses a critical gap in the translation of noninvasive HF RPM research into clinical practice by providing a standardized framework for study design and reporting. By facilitating more consistent and comprehensive reporting of RPM interventions, this framework enables meaningful comparison across studies and allows future research to replicate and iteratively build on prior RPM interventions and findings. With its emphasis on clarity and applicability, this framework is well-positioned to serve as a practical tool for accelerating the integration of effective RPM strategies into diverse routine HF care settings.

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KEY WORDS heart failure, remote patient monitoring, trial reporting, workflow

APPENDIX For supplemental tables and references, please see the online version of this paper.